BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

Check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Blood / organ function	FBC, renal, liver baseline	Review results before AC	FBC / renal / liver each cycle; weekly taxane checks	Confirm recovery before restart / transition	Investigate persistent problems
Heart / nerves	ECHO/MUGA if >65 or indicated; doxorubicin LVEF >50%	Review symptoms / anthracycline exposure	Cardiac review as indicated; taxane neuropathy	Review cumulative dose and symptoms	Report new cardiac symptoms
Support changes	Agree growth-factor product / days	G-CSF required during dose-dense AC	Switch to weekly-paclitaxel premedicines	Do not carry AC support across automatically	Review continuing support needs

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00485 v6 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up

THE WHOLE TREATMENT JOURNEY

Dose-dense AC → weekly paclitaxel: treatment and checks

Adjuvant or neoadjuvant high-risk node-negative or node-positive breast cancer

Four AC treatments (doxorubicin 60 mg/m² and cyclophosphamide 600 mg/m²) 14 days apart, followed 14 days after the last AC by 12 weekly paclitaxel 80 mg/m² doses, subject to recovery. The planned sequence spans about 20 weeks.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Treatment plan NCCP 00485	Eligibility and cardiac risk	AC q14 days ×4	Paclitaxel weekly ×12	Agree local/endocrine treatment	Early-breast follow-up
Blood / organ function NCCP 00485	FBC, renal, liver baseline	Review results before AC	FBC / renal / liver each cycle; weekly taxane checks	Confirm recovery before restart / transition	Investigate persistent problems
Heart / nerves NCCP 00485	ECHO/MUGA if >65 or indicated; doxorubicin LVEF >50%	Review symptoms / anthracycline exposure	Cardiac review as indicated; taxane neuropathy	Review cumulative dose and symptoms	Report new cardiac symptoms
Support changes NCCP 00485	Agree growth-factor product / days	G-CSF required during dose-dense AC	Switch to weekly-paclitaxel premedicines	Do not carry AC support across automatically	Review continuing support needs
Cancer review EARLY	Confirm setting	Agree review plan	Follow the early-breast plan	Next treatment / completion	Agreed surveillance

What each check is for

1 Before treatment

Before the first dose

Adjuvant or neoadjuvant high-risk node-negative or node-positive breast cancer. FBC, kidney and liver tests at baseline and before each cycle, with the unit confirming weekly paclitaxel test requirements. Review current illness and past dose delays separately.

Ask / act: Confirm eligibility, test results and medicine list.

Sources: NCCP 00485

2 Other checks

At the specified review points

Review lifetime anthracycline exposure and cardiac risk. Echo/MUGA is indicated above age 65 or when clinically indicated; the source requires LVEF above 50% for doxorubicin. Reassess cardiac symptoms and neuropathy as indicated; no trastuzumab is included.

Ask / act: Confirm who arranges and reviews each check.

Sources: NCCP 00485

3 Support and practical plan

At each treatment or phase change

G-CSF is required for every dose-dense AC cycle. Prescribe product, dose and timing separately under the approved local plan. First paclitaxel dose: dexamethasone 8 mg IV, chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).

Ask / act: Choose the matching support plan; confirm active medicines.

Sources: NCCP 00485

4 Response and follow-up

Agreed review points

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine CT/PET scans or blood tumour markers are not used to search for distant recurrence in someone without symptoms or clinical concerns. Keep the agreed follow-up and report persistent or new symptoms between visits.

Ask / act: Review symptoms, results and patient priorities.

Sources: EARLY

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.19